

STATION 5: IMMOBILIZATION

HKL'S Sports-Emergency Football Medicine Workshop 2026
Instructor Quick Reference & Participant Handout

PIC	PPP Fakhri / Sr Aznie
Station Equipment	Cervical Collar, Head Immobilizer, Spider Strap, Spinal Board, Scoop Stretcher, Basket Stretcher, Limb Splints, Arm Sling, T-Shirt Arm Sling

1. Principles of Immobilization

- Prioritize life-threatening problems before immobilization.
- Use Manual In-Line Stabilisation (MILS) when cervical spine injury is suspected.
- Choose the device according to the suspected injury, player condition, environment and extraction plan.
- Minimize unnecessary movement during assessment, transfer and extrication.
- Explain the procedure to the player when possible and coordinate all team movements with one clear leader.
- Assess distal circulation, sensation and movement before and after limb immobilization.
- Reassess airway, breathing, circulation and neurological status after immobilization and after every transfer.

REMEMBER: ASSESS → STABILISE → IMMOBILISE → SECURE → REASSESS

2. Cervical Collar

Purpose / Indications

- Restricts excessive cervical spine movement when the cervical spine cannot be clinically cleared.
- Consider when there is cervical midline tenderness, neurological deficit, altered mental status, intoxication or a distracting injury.

Application

- Maintain MILS throughout the procedure.
- Assess the neck and select the correct collar size.
- Prepare the collar before application.
- Position the posterior section and support the chin in the chin piece.
- Secure the collar firmly without excessive tightening.
- Check that the collar does not compromise the airway or breathing.
- Continue appropriate spinal motion restriction and reassess neurological status.

REMEMBER: MILS → ASSESS → SIZE → APPLY → SECURE → REASSESS

3. Head Immobilizer

Purpose

- Provides lateral head support during spinal motion restriction and transport.
- Used together with an appropriate transfer/extrication device when indicated.

Application

- Maintain manual head stabilization until the head is fully secured.
- Position the head in a neutral position unless movement causes pain, resistance or neurological symptoms.

- Place the lateral head blocks/supports evenly.
- Secure the forehead and chin straps according to the device design.
- Ensure access to the airway and ears remains possible.
- Reassess airway and neurological status.

REMEMBER: MANUAL CONTROL → BLOCKS → STRAPS → AIRWAY CHECK → REASSESS

4. Spinal Board & Spider Strap

Purpose

- Provides a rigid surface for controlled extrication or transfer when required.
- Spider straps secure the torso, pelvis and lower limbs to the board.

Application

- Coordinate the movement with one team leader and maintain spinal alignment.
- Position the player centrally on the board.
- Secure the torso and pelvis before the head whenever possible, then secure the legs.
- Apply straps snugly enough to control movement without impairing breathing or circulation.
- Secure the head last while manual stabilization is maintained.
- Reassess ABC and neurological status after securing.

REMEMBER: BODY FIRST → LEGS → HEAD LAST → REASSESS

5. Scoop Stretcher

Purpose

- Allows lifting and transfer with reduced rolling or unnecessary movement.
- Useful for moving an injured player from the pitch to another transport device.

Application

- Set up below the feet, in line with the player (FIFA); adjust the stretcher length to the player.
- Separate the two halves.
- Position each half alongside the player with minimal movement.
- Connect the head and foot ends securely.
- Check all locking mechanisms before lifting.
- Use sufficient rescuers and lift on the team leader's command.
- Reassess the player after transfer.

REMEMBER: SIZE → SPLIT → POSITION → LOCK → CHECK → LIFT

6. Basket Stretcher

Purpose

- Provides a protected carrying platform for difficult terrain or longer manual carries.
- May be useful when pitch access, stairs, slopes or environmental conditions make standard transfer difficult.

Application

- Basket below the feet, in line with the player, 2–3 lengths away; transfer the player in on the scoop or board.
- Position the player centrally.
- Secure the torso, pelvis and legs using the available straps.
- Ensure all straps and handles are secure before movement.
- Use adequate personnel and coordinated commands during lifting and carrying.
- Continuously monitor the player's condition.

REMEMBER: TRANSFER → CENTRE → SECURE → CHECK → CARRY → MONITOR

7. Limb Splints

Indications

- Suspected fracture or significant limb injury requiring movement restriction and support.

Application

- Expose and inspect the injured limb where appropriate.
- Control significant bleeding before splinting.
- Assess distal circulation, sensation and movement before application.
- Support the injury and immobilize the joints above and below the suspected fracture when practical.
- Pad voids and bony prominences as required.
- Secure the splint without excessive pressure.
- Reassess distal circulation, sensation and movement after application.

REMEMBER: CSM BEFORE → SPLINT → SECURE → CSM AFTER

8. Arm Sling

Purpose / Application

- Supports an injured upper limb and reduces painful movement.
- Support the injured arm while applying the sling.
- Position the forearm comfortably across the chest.
- Place the sling under the forearm and secure it around the neck.
- Keep the hand visible when possible for circulation checks.
- Reassess distal circulation, sensation and movement.

REMEMBER: SUPPORT → SLING → SECURE → CSM CHECK

9. T-Shirt Arm Sling

Improvised Technique

- Useful as temporary support when a conventional sling is unavailable or when rapid pitch-side support is required.
- Support the injured arm against the torso in a comfortable position.
- Use the lower part of the player's T-shirt to support the forearm.
- Secure the shirt safely without restricting breathing or circulation.
- Avoid unnecessary manipulation of a suspected fracture or dislocation.
- Reassess distal circulation, sensation and movement.

REMEMBER: SUPPORT → FOLD/SECURE → COMFORT → CSM CHECK

10. Football Pitch-Side Scenarios

Scenario A - Suspected Cervical Spine Injury

- Player has a significant collision and remains on the ground with neck pain or neurological symptoms.
- Approach safely and perform the primary survey.
- Initiate MILS.
- Do not allow the player to sit or stand unnecessarily.
- If the cervical spine cannot be cleared, apply appropriate spinal motion restriction and plan controlled extrication.
- Continue repeated ABC and neurological reassessment.

Scenario B - Suspected Lower-Limb Fracture

- Player has severe limb pain, swelling or deformity after a tackle.
- Perform primary survey and control bleeding if present.
- Assess distal circulation, sensation and movement.
- Support and splint the limb.
- Recheck distal circulation, sensation and movement.
- Scoop stretcher + basket for removal from the pitch – both set up below the feet, in line with the player.

Scenario C - Upper-Limb Injury

- Player has an isolated shoulder/arm injury and is otherwise stable.
- Assess the injured area and distal neurovascular status.
- Support the limb using an arm sling or T-shirt sling as appropriate.
- Reassess comfort and distal circulation before transfer.

11. Common Mistakes to Avoid

- Applying equipment before completing an appropriate primary assessment.
- Releasing MILS too early in a suspected cervical spine injury.
- Incorrect cervical collar sizing.
- Uncoordinated lifting or transfer without a clear team leader.
- Straps that are excessively loose or excessively tight.
- Moving a suspected fracture unnecessarily to obtain a 'normal' position.
- Failing to check distal circulation, sensation and movement before and after splinting.
- Failing to reassess ABC and neurological status after immobilization or transfer.

12. Instructor Teaching Flow

Step	Teaching Point
1	Identify the injury and explain WHY immobilization is needed.
2	Select the correct equipment.
3	Demonstrate the technique slowly.
4	Repeat using normal team speed and clear commands.
5	Participant performs the skill hands-on.
6	Correct technique and highlight safety points.
7	Finish with reassessment and a short pitch-side scenario.

REMEMBER: ASSESS → STABILISE → IMMOBILISE → SECURE → REASSESS

Workshop alignment: This handout follows the supplied Football Pitch-Side Primary Survey Protocol, including MILS for suspected cervical spine injury, cervical collar use when the cervical spine cannot be cleared, splinting for suspected fractures, scoop stretcher use to minimize movement during handling, and safe extrication.

Training note: Device-specific manufacturer instructions and the workshop's local clinical protocol should take precedence where applicable.